DO YOU KNOW?

Ministry of Health and Family welfare, Government of India observes May 16th as National Dengue Day.

It is observed to create awareness about dengue and to intensify preventive measures for the control of disease in the country.

Prevention of Malaria and Dengue

- Avoid Mosquito bites by using mosquito repellents
- Wear long sleeve shirts, long pants and socks.
- Make sure the room doors and windows are closed.
- Doors and windows can be screened with net to prevent mosquitoes from getting in
- Avoid travelling to mosquito prone areas
- Keep your indoors clean. Aedes mosquitoes live indoors. These mosquitoes are found in dark, cool places, such as in closets, under beds etc
- Water from coolers and other small containers (plastic containers, buckets, used automobile tyres, water coolers, pet watering containers and flower vases) should be removed at least once in a week.
- Patients are advised to stay well hydrated.

DO YOU KNOW?

Difference between Aedes and Anopheles mosquitoes



Aedes



Anopheles

6.4.3. Chronic or Long Duration Fever

Chronic fevers are generally of longer and sustained duration. The patients have a past history of repeated episodes of infection. Chronic fevers have a slow, gradual onset and may be low grade in nature. Examples are Tuberculosis, HIV infection and AIDS

Table 6.4.3
Chronic or Long Duration Fevers

	Tuberculosis	HIV infection
Causative Organism	Mycobacterium tuberculosis	Human Immuno Virus – Retro virus
Mode of transmission	Air droplet infection	STD, Through body fluids, contaminated needles
Clinical symptoms	Wasting of tissues	AIDS wasting syndrome

A) Tuberculosis

Tuberculosis is a chronic infectious disease caused by *Mycobacterium tuberculosis.* The disease primarily affects lungs and also other parts of the body such as bones, joints, lymph glands, intestines, kidney and meninges. Mycobacterium tuberculosis is most commonly transmitted by droplet infection. The other rare routes of transmission are through skin and gastrointestinal tract.

The clinical features of pulmonary tuberculosis include anorexia, wasting of tissues, persistent cough, expectoration, night sweats, pain in the chest and fever.

In acute phase, patient presents with pneumonia with high fever. In chronic phase, patient will have low grade fever accompanied with exhaustion, cough, expectoration and loss of weight. The progression of the disease is slow with gradual worsening of the cough.

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